

NIC CPD Micro-Credential: Diabetes Support & Blood Glucose Monitoring for Professional Caregivers

Issuing organisation: National Institute of Caregivers Nigeria (NIC)

Credential type: Stand-alone Continuing Professional Development (CPD) Micro-Credential

Online duration: 5–6 CPD learning hours

Practical component: Separate supervised competency verification where the learner's role, training, care plan and employer policy permit monitoring or medication assistance

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Essential boundary: Completion of this online CPD does not, by itself, authorise a caregiver to diagnose diabetes, prescribe medication, independently alter doses, initiate insulin therapy or manage diabetes outside the authorised role. Practical blood-glucose monitoring or insulin administration requires approved training, delegation, supervision, employer/facility policy and applicable Nigerian requirements.

1. Course title

NIC CPD Micro-Credential: Diabetes Support & Blood Glucose Monitoring for Professional Caregivers

2. Course overview

This stand-alone CPD introduces diabetes, diabetes in older adults, blood-glucose monitoring, capillary testing, hypoglycaemia, hyperglycaemia, emergencies, medication and insulin awareness, nutrition and hydration support, activity, foot and wound awareness, sick-day awareness, psychosocial support, documentation and escalation.

The course supports caregivers to recognise changes and follow a person's care plan. It does not turn caregivers into prescribers, diagnosticians or diabetes clinicians. Blood-glucose thresholds used in teaching are educational reference points only; individual action follows the person's plan, device instructions and professional guidance.

The curriculum is informed by WHO diabetes information ¹, CDC diabetes, sick-day and hypoglycaemia resources ² ³, IDF diabetic-foot recommendations ⁴ and recognised diabetes-care guidance. Local Nigerian clinical pathways, equipment, referral access and scope requirements must be inserted before publication.

3. Target learners

Professional caregivers, healthcare assistants, nursing assistants, home-care workers, residential-care staff, hospital support workers, community caregivers, elder-care workers, disability-support workers, caregivers supporting chronic illness and existing care professionals seeking CPD.

4. Entry expectations

Learners should understand basic caregiving, communication, personal care, infection prevention, documentation and escalation. No clinical qualification is required for online knowledge completion. Practical verification requires authorised supervision and approved equipment.

5. CPD learning hours

The online component is designed for **5–6 CPD learning hours**, including lessons, visuals, knowledge checks, scenarios, module assessments, final assessment and downloadable resources.

6. Central competency

Provide safe, respectful and person-centred support to people living with diabetes, including appropriate blood glucose monitoring within an authorised scope of practice, while recognising abnormal readings and emergencies, promoting healthy routines, documenting observations accurately and escalating concerns appropriately.

7. Learning outcomes

By completion, learners should be able to:

1. Define diabetes mellitus and explain glucose and insulin at a basic level.
2. Distinguish Type 1, Type 2 and gestational diabetes at awareness level.
3. Explain diabetes in older adults and vulnerable people.
4. Recognise common symptoms, risks and complications.
5. Explain the purpose of blood-glucose monitoring.
6. Identify meters, strips, lancets and continuous-monitoring devices.
7. Explain safe capillary-monitoring principles.

8. Record and report readings appropriately.
9. Recognise hypoglycaemia and hyperglycaemia.
10. Recognise signs of DKA, HHS and other emergencies.
11. Explain medication and insulin awareness without changing treatment.
12. Support nutrition, hydration, activity, foot care and sick-day routines within the plan.
13. Provide psychosocial and communication support.
14. Document observations objectively.
15. Identify urgent referral and escalation situations.
16. Explain caregiver scope and practical competency requirements.

8. Course structure

Module	Title	Lessons	Assessment
1	Understanding Diabetes	1. Glucose and Insulin; 2. Diabetes Types; 3. Symptoms and Complications	6 questions
2	Diabetes in Older Adults and People Receiving Care	4. Ageing and Frailty; 5. Cognition, Senses and Risk; 6. Person-Centred Support	6 questions
3	Blood-Glucose Monitoring	7. Why Monitor?; 8. Equipment and Devices; 9. Monitoring Workflow	6 questions
4	Safe Capillary Blood-Glucose Testing	10. Preparation and Consent; 11. Finger-Prick Safety; 12. Recording and Reporting	6 questions
5	Hypoglycaemia, Hyperglycaemia and Emergencies	13. Hypoglycaemia; 14. Hyperglycaemia and Sick Days; 15. Emergencies and Escalation	6 questions
6	Medication, Insulin, Nutrition, Activity and	16. Medication Awareness; 17.	6 questions

	Daily Support	Nutrition and Hydration; 18. Activity and Psychosocial Support	
7	Foot Care, Complications, Documentation and Professional Practice	19. Feet, Skin and Wounds; 20. Documentation and Safeguarding; 21. Integrated Practice and Verification	6 questions

9. Complete Module 1 — Understanding Diabetes

Lesson 1. Glucose and Insulin

Glucose is a form of sugar used by the body for energy. Insulin is a hormone made by the pancreas that helps glucose move from the blood into cells. Diabetes occurs when the body does not make enough insulin, does not use insulin effectively, or both.

Lesson 2. Diabetes Types

Type 1 diabetes involves little or no insulin production and requires clinical management. Type 2 diabetes involves insulin resistance and/or inadequate insulin production. Gestational diabetes develops during pregnancy. Other categories exist, but caregivers need not memorise every classification.

Lesson 3. Symptoms and Complications

Possible symptoms include increased thirst, frequent urination, fatigue, blurred vision, weight change and recurrent infections. Long-term high glucose can affect eyes, kidneys, nerves, heart and feet. Diagnosis and treatment decisions belong to authorised clinicians.

Scenario: A person is unusually thirsty and tired. **Best response:** Observe, document and advise professional assessment; do not diagnose.

10. Complete Module 2 — Diabetes in Older Adults and People Receiving Care

Lesson 4. Ageing and Frailty

Older adults may experience frailty, reduced appetite, mobility limitations, polypharmacy, vision impairment, reduced sensation, falls risk and social isolation. A person may have several conditions and a complex care plan.

Lesson 5. Cognition, Senses and Risk

Dementia, delirium, vision loss or hearing loss can make symptoms less obvious or communication difficult. Confusion, drowsiness or behaviour change may be relevant but has many possible causes. Do not attribute every change to diabetes; observe and escalate.

Lesson 6. Person-Centred Support

Respect choice, independence, family involvement, language, culture and the person's established plan. Do not shame food choices, weight, readings or treatment difficulties. Link dementia-specific communication to NIC's Dementia Care CPD without duplicating it.

11. Complete Module 3 — Blood-Glucose Monitoring

Lesson 7. Why Monitor?

Monitoring can help the person and authorised care team understand glucose patterns, identify abnormal readings and support treatment decisions. Frequency and timing are individual and follow the care plan.

Lesson 8. Equipment and Devices

Equipment may include a glucose meter, test strip, lancing device, single-use lancet, approved skin preparation where applicable, gauze/cotton, sharps container and record sheet/device. Continuous glucose monitors provide sensor readings and have device-specific limitations; they do not eliminate the need to follow the person's plan.

Lesson 9. Monitoring Workflow

The conceptual sequence is:

Prepare → Identify → Explain → Consent → Hand Hygiene → Prepare Equipment → Perform Test According to Device Instructions → Obtain Reading → Dispose Safely → Record → Report/Escalate.

Exact procedures vary by meter manufacturer. Online completion does not authorise unsupervised testing on another person.

12. Complete Module 4 — Safe Capillary Blood-Glucose Testing

Lesson 10. Preparation and Consent

Confirm identity, request/plan, timing requirement and consent. Check meter, strip compatibility, expiry, storage condition and device instructions. Perform hand hygiene and prepare the sharps container before use.

Lesson 11. Finger-Prick Safety

Use a single-use lancet. Never share lancing devices or reuse lancets. Follow approved site and device instructions, prevent cross-contamination, dispose of sharps immediately and respond to blood exposure according to policy.

Lesson 12. Recording and Reporting

Record date/time, reading and units, relation to meals or activity where required, device/strip issue, symptoms, action and person notified. A reading is not interpreted in isolation; compare with the care plan and report abnormal or unexpected results.

13. Complete Module 5 — Hypoglycaemia, Hyperglycaemia and Emergencies

Lesson 13. Hypoglycaemia

Hypoglycaemia means blood glucose below the person's target or an established low threshold. CDC educational resources commonly use below 70 mg/dL (about 3.9 mmol/L) as low 2, but the person's plan and local guidance take priority.

Possible signs include sweating, shaking, hunger, weakness, dizziness, confusion, irritability, difficulty concentrating, behaviour change and drowsiness. Severe episodes may involve inability to swallow, loss of consciousness or seizures.

If the person is awake and can swallow safely, follow the person's hypo plan and local protocol. Never give food or drink by mouth to someone unconscious or unable to swallow

safely. For severe symptoms, call emergency help, protect the person and follow the emergency plan.

Lesson 14. Hyperglycaemia and Sick Days

Hyperglycaemia is high blood glucose. Possible signs include thirst, frequent urination, fatigue, blurred vision, dehydration, nausea or vomiting. Illness can disrupt glucose control.

Sick-day action follows the person's written plan. Do not independently stop insulin, alter medicines or advise doses. Encourage appropriate fluids only when safe and allowed by the plan, monitor/report as directed and escalate vomiting, dehydration, breathing changes, severe weakness, confusion or deterioration.

Lesson 15. Emergencies and Escalation

DKA and HHS are serious hyperglycaemic emergencies. Caregivers do not diagnose them. Urgent escalation is required for altered consciousness, seizure, severe confusion, repeated vomiting, severe dehydration, breathing difficulty, collapse, very abnormal readings with symptoms, suspected DKA/HHS or any rapid deterioration.

Scenario: A person is unconscious with a low reading. **Best response:** Do not give oral food/drink; call emergency help, protect airway/position according to training and follow the emergency plan.

14. Complete Module 6 — Medication, Insulin, Nutrition, Activity and Daily Support

Lesson 16. Medication Awareness

Diabetes medicines may be oral or injectable; insulin replaces or supplements insulin the body lacks. Caregivers may remind, observe, store or assist only where authorised. Never independently change a prescribed dose, initiate insulin, skip treatment or substitute medication.

Report missed doses, vomiting after medication, suspected side effects, low readings, repeated high readings or inability to follow the plan.

Lesson 17. Nutrition and Hydration

Regular meals, carbohydrate awareness, balanced nutrition, hydration and meal timing may be important. Support the individual plan; do not prescribe a “diabetic diet,” shame

Nigerian foods or assume one food pattern suits all clients. Refer dietary questions to an authorised professional and link to NIC' s Nutrition, Hydration & Mealtime Support CPD.

Lesson 18. Activity and Psychosocial Support

Appropriate activity can support wellbeing, but activity must follow the person' s plan and safety guidance. Consider mobility, foot health, glucose risk, hydration and supervision. Do not prescribe exercise programmes.

Diabetes can create fear, stigma, treatment fatigue and social stress. Listen, encourage, protect autonomy and refer distress or safeguarding concerns.

15. Complete Module 7 — Foot Care, Complications, Documentation and Professional Practice

Lesson 19. Feet, Skin and Wounds

Diabetes can increase risk of reduced sensation, poor circulation, skin damage, ulcers, infection and delayed healing. Observe feet for cuts, blisters, redness, swelling, colour or temperature change, cracks, drainage, footwear problems and new pain. Do not cut corns, remove calluses, diagnose infection or treat wounds independently. Link to NIC' s Wound Care CPD.

Lesson 20. Documentation and Safeguarding

Record readings, timing, symptoms, food/activity context where required, medication assistance, foot/skin observations, care provided, client response, escalation, person notified and follow-up. Use factual language and protect confidentiality.

Safeguarding includes protection from coercion, neglect, financial exploitation, food deprivation, medication misuse and discriminatory treatment. Follow the safeguarding pathway.

Lesson 21. Integrated Practice and Verification

Apply: **identify → explain/consent → prepare → monitor only within scope → observe symptoms → follow plan → record → report → escalate**. Online knowledge and practical competency are separate achievements.

16. Visual learning plan

Visual	Purpose
Glucose–insulin diagram	Explain basic physiology
Diabetes-type comparison	Distinguish Type 1, Type 2 and gestational awareness
Monitoring workflow	Show safe conceptual sequence
Meter-kit layout	Identify equipment and single-use items
Hypo/hyper symptom table	Support recognition without diagnosis
Emergency escalation card	Show severe-symptom response
Sick-day decision tree	Connect plan, monitoring and referral
Foot-observation illustration	Encourage inspection and prompt reporting
Objective documentation example	Teach factual records
Scope decision tree	Separate knowledge from authorised practice

17. Scenario-based learning

1. New thirst and urination: observe and refer; do not diagnose.
2. Older adult suddenly confused: check immediate safety and escalate; do not assume dementia or diabetes.
3. Reading below the person's hypo threshold: follow plan; assess consciousness and swallowing.
4. Unconscious person with low reading: no oral intake; emergency escalation.
5. Repeated high readings and vomiting: urgent professional/emergency review.
6. Meter strip expired: do not rely on result; replace and report.
7. Family asks caregiver to change insulin dose: decline and contact authorised clinician.
8. Client skips meals: report risk and follow care plan; do not shame.
9. New foot blister: protect, avoid self-treatment and escalate.
10. Person distressed by diabetes: listen, support and refer appropriately.

18. Module assessments

Module 1

1. What is glucose? **A fuel used by cells.**
2. What does insulin do? **Helps glucose move into cells.**
3. Name three diabetes categories. **Type 1, Type 2 and gestational.**
4. Name three symptoms. **Thirst, frequent urination, fatigue, blurred vision or weight change.**
5. Who diagnoses diabetes? **Authorised clinician.**
6. Is diabetes a chronic condition requiring ongoing management? **Yes.**

Module 2

1. Name three older-adult risks. **Frailty, reduced appetite, vision loss, mobility limitation, polypharmacy or falls.**
2. Can symptoms be less obvious? **Yes.**
3. What should sudden confusion trigger? **Safety check and escalation.**
4. Should food be shamed? **No.**
5. What supports person-centred care? **Choice, independence and communication.**
6. What course provides deeper dementia communication? **NIC Dementia Care CPD.**

Module 3

1. Why monitor glucose? **To identify patterns and abnormal readings for care decisions.**
2. Name four supplies. **Meter, strip, lancet, lancing device, gauze or sharps container.**
3. Do devices use identical instructions? **No.**
4. State the workflow. **Prepare, identify, explain, consent, hygiene, equipment, test, dispose, record, report.**
5. Can online completion authorise testing? **No.**
6. What must guide frequency? **The care plan.**

Module 4

1. What should be checked on strips? **Compatibility, expiry and storage.**
2. Are lancets single-use? **Yes.**
3. Should lancing devices be shared? **No.**
4. Where do sharps go? **Approved sharps container.**

5. What should be recorded? **Time, reading, units, context, symptoms, action and notification as required.**
6. What if result is unexpected? **Repeat/report according to device and plan; do not conceal.**

Module 5

1. Name four hypo signs. **Sweating, shaking, hunger, weakness, confusion or drowsiness.**
2. What threshold is commonly used educationally? **Below 70 mg/dL/3.9 mmol/L, subject to local plan.**
3. Can an unconscious person receive oral food/drink? **No.**
4. Name four hyperglycaemia signs. **Thirst, urination, fatigue, blurred vision, vomiting or dehydration.**
5. What conditions require urgent escalation? **Seizure, altered consciousness, vomiting, severe dehydration, breathing changes or collapse.**
6. Can caregivers diagnose DKA/HHS? **No.**

Module 6

1. Can caregivers alter insulin doses? **No, unless explicitly authorised under the applicable framework.**
2. What if medication is missed? **Report and follow the plan.**
3. Can caregivers prescribe a diet? **No.**
4. Name two nutrition-support actions. **Support plan, encourage appropriate fluids/meals and report poor intake.**
5. Can caregivers prescribe exercise? **No.**
6. What psychosocial support is appropriate? **Listening, encouragement and referral.**

Module 7

1. Name three diabetic-foot concerns. **Cuts, blister, redness, swelling, colour change, drainage or new pain.**
2. Should corns be cut independently? **No.**
3. What belongs in documentation? **Reading, symptoms, action, response, escalation and notification.**

4. Name two safeguarding risks. **Coercion, neglect, medication misuse, food deprivation or discrimination.**
5. Does documentation replace escalation? **No.**
6. Does online completion prove clinical diabetes competence? **No.**

19. 40-question final assessment

Recommended pass mark: 80% (32/40). At least 30 questions are scenario/application based.

1. **MCQ:** Insulin primarily helps: A) glucose enter cells; B) wounds close; C) vision improve; D) bones grow. **Answer: A.**
2. **Scenario:** New thirst and frequent urination. **Answer:** Observe/report for professional assessment; do not diagnose.
3. **True/False:** Type 1 and Type 2 diabetes are identical. **Answer: False.**
4. **Application:** Name three diabetes symptoms. **Answer:** Thirst, urination, fatigue, blurred vision or weight change.
5. **Scenario:** Older adult has new confusion. **Answer:** Check safety and escalate; do not assume dementia.
6. **MCQ:** Caregiver authority depends on: A) confidence; B) scope, training, policy and supervision; C) family request; D) internet advice. **Answer: B.**
7. **Scenario:** Family asks caregiver to change insulin. **Answer:** Decline and contact authorised clinician.
8. **True/False:** Diabetes requires ongoing management. **Answer: True.**
9. **MCQ:** Monitoring frequency follows: A) care plan; B) guesswork; C) social media; D) caregiver preference. **Answer: A.**
10. **Scenario:** Expired strips are found. **Answer:** Do not rely on them; replace/report.
11. **Application:** Name five monitoring supplies. **Answer:** Meter, strips, lancing device, single-use lancet, gauze and sharps container.
12. **True/False:** Lancing devices may be shared between clients. **Answer: False.**
13. **Scenario:** Learner wants to practise on a friend. **Answer:** Do not practise outside approved supervised training.
14. **MCQ:** The workflow ends with: A) record/report; B) diagnosis; C) dose change; D) disposal only. **Answer: A.**

15. **Scenario:** Result is unexpectedly high. **Answer:** Follow device/plan repeat and reporting process; do not independently treat.
16. **MCQ:** Educational hypoglycaemia reference is commonly below: A) 70 mg/dL; B) 300 mg/dL; C) 500 mg/dL; D) no value. **Answer: A, subject to individual plan.**
17. **Scenario:** Person is sweating, shaking and alert. **Answer:** Follow the person's hypo plan and local protocol.
18. **Scenario:** Person is unconscious. **Answer:** No oral food/drink; emergency escalation and protect according to training.
19. **True/False:** Confusion may be a hypoglycaemia sign. **Answer: True.**
20. **Scenario:** Seizure occurs. **Answer:** Protect, call emergency help and follow emergency plan.
21. **Application:** Name four hyperglycaemia signs. **Answer:** Thirst, urination, fatigue, blurred vision, vomiting or dehydration.
22. **Scenario:** High reading with vomiting and deep/abnormal breathing. **Answer:** Urgent emergency escalation; do not diagnose or dose-adjust.
23. **True/False:** Caregivers independently diagnose DKA. **Answer: False.**
24. **Scenario:** Person with diabetes is ill and not eating. **Answer:** Follow sick-day plan, report promptly and do not stop/change medication independently.
25. **MCQ:** Insulin doses may be changed by caregivers: A) whenever readings are high; B) only when explicitly authorised under framework; C) after family request; D) never by anyone. **Answer: B.**
26. **Scenario:** Missed diabetes medicine. **Answer:** Record/report and follow authorised instructions.
27. **Scenario:** Client is ashamed of traditional food. **Answer:** Avoid food shaming; support individual plan and refer dietary questions.
28. **True/False:** One Nigerian diet should be prescribed for every person with diabetes. **Answer: False.**
29. **Scenario:** Person wants exercise but has a foot blister. **Answer:** Follow plan, protect foot and seek professional guidance.
30. **MCQ:** Diabetic-foot observation includes: A) cuts and colour change; B) cutting calluses; C) prescribing antibiotics; D) probing wounds. **Answer: A.**
31. **Scenario:** New foot drainage. **Answer:** Protect, document and promptly escalate.
32. **True/False:** Caregivers should remove calluses independently. **Answer: False.**

33. **Application:** Name five documentation fields. **Answer:** Reading/time, symptoms, context, action, response, escalation and notification.
34. **Scenario:** Caregiver writes “non-compliant.” **Answer:** Replace with objective facts.
35. **Scenario:** Family withholds meals as punishment. **Answer:** Recognise safeguarding concern and report through pathway.
36. **True/False:** Documentation replaces emergency escalation. **Answer: False.**
37. **Scenario:** Client with dementia cannot explain symptoms. **Answer:** Use adapted communication, observe carefully and escalate change.
38. **MCQ:** Practical monitoring competence requires: A) online reading; B) supervised assessment; C) confidence; D) attendance. **Answer: B.**
39. **Scenario:** Caregiver is unsure whether a reading requires action. **Answer:** Do not improvise; follow plan and contact authorised professional.
40. **Integrated scenario:** A client has reduced intake, a low reading, confusion and a caregiver who wants to give food immediately. **Answer:** Assess immediate safety and consciousness, do not give oral intake if unable to swallow safely, follow emergency/hypo plan, escalate urgently and document.

20. Answer key

Answers are embedded in the assessment. The central principles are: support rather than independently manage; follow the care plan; use single-use equipment; recognise symptoms in context; never give oral intake to someone unable to swallow safely; never alter medication or insulin without authority; protect feet and dignity; document objectively; and escalate uncertainty or emergency signs.

21. Practical competency verification framework

Practical verification is separate from online completion and must be limited to authorised tasks.

Station	Capability
Identity and consent	Confirms person, plan, explanation and consent
Equipment	Checks meter/strips, expiry, compatibility and sharps readiness
Infection prevention	Hand hygiene, single-use lancet, cleaning and cross-contamination prevention

Capillary monitoring simulation	Performs approved device process under supervision
Reading and response	Records result and follows plan without independent interpretation beyond scope
Hypo response	Recognises symptoms, consciousness/swallowing status and escalation need
Foot observation	Identifies/reportable changes without treatment or diagnosis
Documentation	Records time, reading, context, symptoms, action and notification
Safeguarding and communication	Maintains dignity, choice, privacy and respectful language

Critical failures: sharing lancets or devices; unsafe sharps disposal; testing without consent; changing medication/insulin; giving oral intake to an unconscious person; ignoring severe symptoms; diagnosing DKA/HHS; treating a foot wound independently; breaching confidentiality; or signing off an unobserved skill.

22. Downloadable resources

1. Diabetes scope-of-practice decision tree.
2. Blood-glucose monitoring preparation checklist.
3. Meter and strip compatibility worksheet.
4. Capillary-testing safety card.
5. Reading and reporting record.
6. Hypoglycaemia recognition and emergency card.
7. Hyperglycaemia and sick-day escalation card.
8. Medication and insulin awareness guide.
9. Nutrition, hydration and activity support checklist.
10. Diabetic-foot observation guide.
11. Diabetes documentation template.
12. Practical competency checklist.

23. Certificate recommendation

Online CPD certificate

This certificate recognises successful completion of NIC online CPD in diabetes support and blood-glucose monitoring principles, including diabetes awareness, older-adult considerations, capillary monitoring, hypo/hyperglycaemia recognition, emergency escalation, medication and insulin awareness, nutrition, hydration, activity, foot care, psychosocial support, documentation and scope of practice. **This certificate represents online knowledge completion only. It does not authorise diagnosis, prescribing, insulin initiation, dose changes, medication changes or independent clinical diabetes management. It is not a nursing, medical or diabetes-specialist qualification, professional licence or automatic statutory workplace certification.**

Practical endorsement

Issued only after supervised verification of named skills by an authorised assessor, with date, equipment, protocol, result, remediation and reassessment. It does not expand legal or employment scope.

24. Digital badge recommendation

NIC Diabetes Support & Blood Glucose Monitoring CPD — Knowledge Completion

Field	Specification
Description	Online CPD knowledge in safe, respectful diabetes support
Competencies	Diabetes awareness, monitoring principles, hypo/hyper recognition, foot awareness, documentation and escalation
CPD hours	5–6 online hours
Evidence	Modules, assessments, final score $\geq 80\%$, scope acknowledgement
Limitation	No prescribing, dose adjustment, insulin authorisation or clinical licence
Review	Recommend review every 24 months or when guidance changes

25. Portal implementation notes

Use **Course → Modules → Lessons → Knowledge Checks → Module Assessments → Final Assessment → Certificate**. Track online completion, score, scope acknowledgement, practical verification status, assessor evidence, badge status and review date separately.

26. Nigerian context and scope-of-practice notes

Hospitals, clinics, residential care, home care and community settings differ in equipment, strips, electricity, refrigeration, transport, emergency response, nutrition access, referral pathways and clinical supervision. NIC should require each employer/facility to insert current monitoring SOP, meter instructions, units used, emergency contacts, hypo plan, sick-day pathway, medication-assistance policy, foot-referral route and authorised caregiver activities.

Do not invent Nigerian laws or clinical requirements. Verify Nigerian regulatory and scope claims against current authoritative Nigerian sources and the employer's governance process before publication. Avoid stereotyping Nigerian foods or prescribing one cultural diet.

27. References

- [1] World Health Organization, Diabetes fact sheet.
- [2] Centers for Disease Control and Prevention, Treatment of Low Blood Sugar.
- [3] Centers for Disease Control and Prevention, Managing Sick Days with Diabetes.
- [4] International Diabetes Federation, Clinical Practice Recommendations on the Diabetic Foot.
- [5] Centers for Disease Control and Prevention, Promoting Foot Health.

28. Quality control review

Review area	Finding	Required action
Clinical accuracy	Diabetes concepts, monitoring, hypo/hyperglycaemia and foot awareness are covered at caregiver level.	Diabetes educator review before launch.
Patient safety	Emergency boundaries, consciousness/swallowing	Add local emergency contacts and plans.

	warning and medication limits are prominent.	
Scope	Diagnosis, prescribing, dose changes and independent management are excluded.	Governance sign-off.
Emergency safety	DKA/HHS are awareness topics with urgent escalation, not caregiver diagnoses.	Validate local emergency pathway.
Nigerian relevance	Resource variation, cultural food practices and local SOP insertion are addressed.	Verify local claims.
Assessments	40 questions with scenario/application emphasis are included.	Pilot and item-analyse.
Practical competence	Separate supervised framework and critical failures are provided.	Approve assessors and equipment.
Duplication	Links to Nutrition, Wound Care, Vital Signs, Medication, First Aid and Dementia CPDs are clear.	Add portal cross-links.
Accessibility	Plain language, tables, scenarios and job aids support varied learners.	Pilot with caregivers.
Visual quality	Visual plan supports understanding without unsafe treatment instruction.	Clinical and safeguarding review.

Final learner message: “I understand diabetes, I can support a person living with diabetes safely, I can recognise important changes and emergencies, I can monitor and report appropriately where my role permits, and I know when to escalate.”